must not rest solely on the presence of "retinal hemorrhage" in either the clinical or the research setting.[2] In addition to describing the type, location, distribution pattern, and number of retinal hemorrhages, one must also take into consideration the history (or absence of history) and the nature of other injuries, particularly to the bones and brain.

The medical problem of child abuse is discussed in a unique medicolegal context, so much so that some physicians have virtually made their careers out of testifying as expert witnesses. Legal and medical definitions of science and fact can be disturbingly different. Clarity of definition and adherence to proper medical scientific principles of investigation, including peer-review publication, remain an essential element in exploring and understanding the pathophysiology and potential forensic implications of findings such as retinal hemorrhage.

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Macular Hole Surgery

Dear Editor:

We greatly appreciate the reply to our letter by Dr. Da Mata and colleagues[1] and fully agree that more comparative data on functional outcome of indocyanine green (ICG)–assisted macular hole surgery are necessary. Therefore, our clinical and histopathologic results have been submitted for publication.

From a clinicopathologic point of view, one comment by Dr. Da Mata and colleagues needs further clarification. They used the term "surface of retinal ILM," which might be misleading, as the interpretation of histologic findings requires further information on which side of the internal limiting membrane (ILM) the authors refer to, the retinal or the vitreal side. When evaluating the ultrastructure of the ILM removed during vitrectomy, the key questions are the following:

1. Are cellular elements (such as inner portions of Müller cells) present on the retinal surface of the ILM?
2. If so, can this be attributed to ILM peeling itself or to the additional use of ICG?

In their reply Dr. Da Mata and colleagues mention the presence of cellular elements "on the surface of retinal ILM" in eyes with "coexisting epiretinal membranes." In these cases, it should not be surprising to find cellular elements on the vitreal side of the ILM, as shown by previous reports as well as by Dr. Da Mata and coworkers themselves.[2]

Regarding the retinal side of the ILM, there were no cellular elements present after ICG-assisted peeling, as shown in Figure 2 of the latter report.[2] In our experience, the absence of cellular elements on the retinal side of the ILM is consistent with ultrastructural analysis of specimens after ILM peeling, but *without* ICG assistance. In contrast, after ICG-assisted ILM removal, cellular elements of the inner retina adherent to the ILM were common features of histologic specimens.[3] In a previous study these findings were also observed in cadaver eyes but were considered a postmortem artifact.[4] The presence of cellular structures adherent to the ILM was further reported by Kwok and colleagues after ICG-guided vitrectomy.[5] However, from the figures provided in their report and due to the histologic technique applied, it was hardly possible to determine the real location of these cells.[6]

In summary, we emphasize that retinal structures had been found on the retinal surface of the ILM after ICG-assisted vitrectomy not only in cadaver eyes but also after ICG-guided macular hole surgery in humans. Attention should be paid to ultrastructural analysis of ILM specimens after ICG-assisted peeling, as this might further clarify recent controversial discussions on possible negative effects of ICG on the human retina.

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Postoperative Wound Dehiscence

Dear Editor:

The article by Abou-Jaoude et al[1] brings to mind a postoperative complication after penetrating keratoplasty (PKP) that is rarely investigated. In this retrospective, noncomparative study of 69 eyes that underwent removal of a single running suture after PKP, the authors identified 5 that de-